# Exemestane · daily hormone therapy

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

| Check | Before starting | First weeks | During treatment | Longer-term review | Change / completion |
|---|---|---|---|---|---|
| **Bloods / symptom review** | FBC, renal and liver profile | Review tolerability and medicines | Further tests when clinically indicated | Review symptoms, adherence and results | Follow unresolved abnormalities |
| **ECG / echo** | No routine ECG / echo for this medicine alone | Investigate new cardiac symptoms | Further cardiac tests by risk / symptoms | Companion-drug cardiac rules remain separate | Follow abnormalities when indicated |
| **Bone density / DXA** | Adjuvant AI: baseline DXA unless already on adjuvant bisphosphonates | Review previous DXA and fracture risk | No cycle-based DXA appointments | Lower-risk AI: reassess in 1–2 years; individualise | Continue bone care according to risk |
| **Early-breast surveillance** | Confirm early / curative setting | Set first follow-up mammogram date | Assess new symptoms promptly | Annual mammography of remaining breast tissue | No routine CT / markers if asymptomatic |
| **Advanced-disease response** | Document disease sites / baseline assessment | New symptoms can bring assessment forward | Generally every 2–4 months; individualise | Adapt interval and imaging to disease / benefit | Markers supplement, never replace, assessment |

THE WHOLE TREATMENT JOURNEY

# Exemestane · daily hormone therapy: the whole treatment pathway

**Postmenopausal ER-positive breast cancer after prior endocrine treatment**

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First weeks | During treatment | Longer-term review | Change / completion |
|---|---|---|---|---|---|
| **Early: surgery / chemo** ESMO_EBC2024 | Surgery / pathology guide the plan | Any indicated chemotherapy first | Start endocrine after chemotherapy | Review recovery and lasting effects | Agree next endocrine decision |
| **Early: radiotherapy** ESMO_EBC2024 · EVIQ_RT3650 | Decide if radiation is indicated | Usually after surgery and any chemo | AI may overlap; tamoxifen timing agreed | Complete RT / review local effects | Continue endocrine beyond RT if planned |
| **Early: endocrine medicine** NCCP 00376 | Confirm setting and prior treatment | Start daily tablets at agreed point | Continuous medicine between reviews | Review planned total course / extension | A review is not an automatic stop |
| **Advanced: medicine** NCCP 00376 | Confirm exact advanced indication | Daily tablets | Continue if beneficial | Review cancer control and tolerance | Agree next treatment / care |
| **Advanced: local treatment** ESMO_MBC2023 | Define any local-control / symptom need | Surgery / radiation only if indicated | Teams agree overlap or sequence | Review symptoms and local effects | Reassess goals if disease changes |
| **Safety checks** NCCP 00376 · SMPC | Baseline FBC, renal and liver profile | Review symptoms / interactions | Bloods when clinically indicated | Risk-based cardiac / other checks | Follow unresolved abnormalities |
| **Scans / bone health** NICE_NG101 · ESMO_EBC2024 · ESMO_MBC2023 | Baseline disease and fracture assessment | Confirm which setting-specific rows apply | Keep agreed scan / DXA appointments | Early: mammography; advanced: response scans | Name who books and reviews results |

# What each check is for

**1 Bloods and symptoms**

Baseline FBC, renal and liver profile; later blood tests are clinically indicated, not automatic chemotherapy-cycle tests. Review symptoms, adherence, medicines and relevant menopause uncertainty.

Sources: NCCP 00376

**2 Heart monitoring**

There is no routine ECG or echo schedule for exemestane alone. Investigate symptoms and manage cardiovascular risk when indicated.

Sources: NCCP 00376 · SMPC

**3 Bone health**

Adjuvant AI: offer baseline DXA unless already receiving adjuvant bisphosphonates. T-score above −2.0 with no additional risk factors: reassess risk / BMD every 1–2 years. On bone-protective treatment: generally about every 2 years. Fracture, substantial bone loss or changed risk may need earlier review; the team individualises the plan.

Sources: NICE_NG101 · AIBL2025

**4 Cancer scans**

Early / curative: annual mammography of remaining breast tissue; no routine mammography on the mastectomy side, CT, bone scans or tumour markers if asymptomatic. Advanced: response assessment generally every 2–4 months, adapted to disease pace and symptoms; less often may suit indolent disease. The team selects imaging, not automatically CT chest/abdomen/pelvis. New symptoms can bring assessment forward.

Sources: ESMO_EBC2024 · ESMO_MBC2023

**5 Markers**

Tumour markers may supplement imaging and clinical assessment in advanced disease if they are informative, especially if elevated at baseline. They do not replace scans, and a marker change alone should not determine treatment failure. There is no universal fixed marker timetable.

Sources: ABC5_MARKERS

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.